



Care coordination in mental health

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Functional sub group	Mental health
Summary	Care coordination is the process for helping people experiencing mental health concerns to access a range of different services to support their recovery journey.
Policy owner	Executive Director Mental Health
Contact	Carole Steiner - 08 9431 2013
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NSQHS Standards	Standard 1: Clinical Governance Standard 2: Partnering with Consumers Standard 5: Comprehensive care Standard 6: Communicating for Safety Standard 8: Recognising and Responding to Acute Deterioration
Office Chief Psychiatrist Standards for Clinical Care	Standard 1: Aboriginal Practice Standard 2: Assessment Standard 3: Care planning Standard 4: Consumer and carer involvement in individual care Standard 5: Physical health care of mental health consumers Standard 6: Risk Assessment and management Standard 7: Transfer of Care
Status	Active

1. Background

1.1. About this document

This document outlines the key elements of care coordination and describes the roles and responsibilities of all mental health clinicians who are involved in coordinating care for mental health consumers within the South Metropolitan Health Service (SMHS). The document provides overarching governance for SMHS mental health services, including statewide services and should be read in conjunction with relevant legislation, standards and policies.

1.2. Key definitions

Term	Definition
Care Coordinator	The allocated member of the multidisciplinary team (MDT) who has the responsibility for coordinating, facilitating and integrating mental health treatment, care and support to a mental health consumer and their family / significant others.
Evidence based care	A process of integrating the best available research evidence with clinical expertise and the consumers unique values and circumstances. It requires the health professional to consider characteristics of the practice context in which they work.
Multidisciplinary Team (MDT)	A team of mental health staff usually comprised from a range of disciplines including psychiatry, psychology, nursing, occupational therapy, social work, pharmacy, peer workers, welfare officers, Aboriginal Mental Health Liaison and other relevant groups who collaborate in the assessment, planning and delivery of care. The MDT supports the Care Coordinator in taking a holistic approach to the needs of the person accessing care, including physical, social, cultural, emotional and spiritual wellbeing, relevant to the different disciplines.
Person centred care	An approach to care delivery where the person is treated respectfully as an individual human being, and not just as an illness or condition to be treated. It involves seeking out and understanding what is important to the person, their families, carers and support people, fostering trust and establishing mutual respect. It also means working together to share decisions and plan care.
Recovery	The achievement of an optimal state of personal, social and emotional wellbeing, as defined by each individual, whilst living with or recovering from a mental health issue.
Recovery oriented practice	An approach to care delivery that emphasises the importance of understanding that each person is different and should be supported to make their own choices, listened to and treated with dignity and respect. Each person is the expert of their own life and support should assist them to achieve their hopes, goals and aspirations. Recovery will mean different things to different people. Refer to Recovery focused care and language .

Relevant service providers	Service providers outside of the treating health service who provide physical or mental health care or psychosocial supports, including but not limited to: • Non-Government Organisations • Community Controlled Organisations • Aboriginal Community Controlled Organisations • National Disability Insurance Scheme (NDIS) Registered Providers • General Practitioners • Private psychiatric hostels
Support people	People nominated by the person accessing care includes family members, carers, nominated person, parent, guardian of a child or enduring guardian of an adult.
Trauma informed practice	A model that is grounded in and directed by a complete understanding of how trauma exposure affects a person's neurological, biological, psychological and social development. The practice is underpinned by principles of safety, trustworthiness, choice, empowerment and collaboration.

2. Policy Statement

Care coordination is a person centred, assessment based, recovery-focused, interdisciplinary approach to integrating health care and support in which an individual's needs and preferences are assessed, a comprehensive Treatment Support and Discharge Plan is collaboratively developed, and services are coordinated and monitored by an allocated Care Coordinator. Evidence indicates that care coordination improves experience of care, promotes a recovery approach, improves quality of life and delivers better health outcomes.

All people accessing mental health services within SMHS will have access to care coordination through appropriate services that support their care from the point of entry or re-entry to discharge. Entry to community mental health services will be based on referral, triage and assessment. The Care Coordinator (or allocated clinician within the MDT) will ensure that appropriate contact, coordination and communication are maintained throughout any inpatient admission to ensure safe transition between care settings.

SMHS is committed to providing a safe and supportive working environment to SMHS staff, who undertake their practice in all setting types including inpatient and community mental health services and the consumers' place of residence. SMHS also acknowledges the duty of care to provide optimal support and risk mitigation strategies to staff where there is an awareness of pre-identified or anticipated risk factors, with a potential threat to the safety of the staff and/or the consumer and/or family/significant others.

Refer to [SMHS Community / Home Visiting policy](#)

3. Guiding principles and practices

The following principles and practices underpin care coordination:

- Person centred

- Recovery oriented
- Trauma informed
- Socially inclusive and supportive of diversity in all forms including cultural, sexual, gender and/or bodily diversity.
- Evidence based
- Collaborative partnering
- Governance, accountability and scope of practice according to service Models of Care.

Refer to Appendix A. SMHS MH Care Coordination Procedure.

4. Consent

At the time of assessment:

- The person accessing care should be advised that it may be helpful to their care for the assessor:
 - to access their information held by relevant service providers or health care providers.
 - to share their information or discuss their circumstances with other team members, agencies or their carer.
- Informed consent from the consumer should be obtained to share information with other agencies and support persons promptly after admission to the service. It must be recorded in the patient's medical record.
- Where the consumer declines consent, the clinician will explore and document the consumers rationale for refusal. The consumers refusal to give consent and potential risk issues will be considered, discussed with the senior clinician and the MDT where appropriate, and a plan clearly documented regarding safety of the individual, carer and others.

Refer to [WA Health Consent to Treatment Policy](#).

5. Allocation of a Care Coordinator

Once the required referral /entry information and appropriate consent have been obtained and the person is activated into the relevant program, a Care Coordinator will be allocated.

The selection of a care coordinator for the person receiving care will be based on the following:

- The person's needs, complexity and potential requirements including Community Treatment Orders.
- Consideration of relevant background factors, including culture, diversity, sexual, gender and/or bodily diversity.
- The Care Coordinators professional qualification, knowledge, skills and experience align with the persons needs and supports their journey. Identifying a 'best fit' Care Coordinator supports in building and maintaining the therapeutic relationship.
- The current caseload of the Care Coordinator, considering acuity levels and volume.
- In the event the persons need change significantly or they have expressly requested a change to the allocated Care Coordinator, this should be considered through escalation and discussion in the MDT.
- Where a person re-enters the service and is accepted by the same team, the previously allocated Care Coordinator should be considered, in addition to the above criteria.

- A nominated member of the MDT will be allocated the role of Care Coordinator during periods of absence of the primary Care Coordinator.

6. Roles and Responsibilities

6.1. MDT

The MDT imbues/enables/facilitates/upholds collaboration in the assessment, planning and delivery of mental health services. It enables both core and specialist assessments and interventions to be provided in an integrated manner in collaboration with the person receiving care, support people and other relevant service providers. Whilst there will be a single allocated Care Coordinator, it is recognised that the task of providing comprehensive services is beyond the capacity of one professional.

There are both shared tasks and specialist tasks in the coordination of care, and these are performed across the MDT membership and within specific disciplines respectively. The MDT must consider involving Aboriginal Mental Health Liaison or Aboriginal Liaison Officers where appropriate and available.

The role of the MDT in care coordination is to, collaboratively:

- participate in the decision regarding the allocation of the Care Coordinator
- participate in discussion and decision regarding the level of assertiveness required to engage and involve the person in their treatment and support particularly where engagement is an issue
- participate in regular and ad hoc clinical reviews led by the Care Coordinator, to ensure that specialist skills and advice are optimised to meet requirements, including escalation if needs change
- participate in the discharge planning
- participate in the re-allocation of the Care Coordinator if it is indicated an alternate skill set is required for an extended duration of time to best support the person's needs.

6.2. Care Coordinator

The Care Coordinator, in collaboration with the MDT, is responsible for tailoring care and support to address the specific needs of the person receiving care and their support people. This includes facilitating and integrating mental health assessment, treatment and support that is person centred and evidence-based and meets the requirements of all relevant legislation, standards and policies.

7. Care Coordinator – key functions

Key functions of the Care Coordinator include:

7.1. Promoting engagement, involvement and choice

The Care Coordinator promotes engagement and choice at all possible times for the person who is accessing the service, to support them to be actively involved in their care. The Care Coordinator will support the person to be actively involved in their own care and treatment by:

- Creating an effective therapeutic relationship with the person, emphasising trust and collaboration to support their journey in a way that is meaningful.
- Holistic assessment including mental, physical, psycho-social, cultural, and sexual, gender and/or bodily diversity needs.
- Assisting the person in identifying what matters to them, their personal and recovery goals, individual strengths, and perceived obstacles that should be considered in their Treatment, Support, and Discharge Plan (TSDP).
- Considering choices, options, and preferences to promote and facilitate a recovery approach.
- Providing an inclusive service and ensuring appropriate actions are taken to engage other services, e.g., interpreters, linking with cultural groups or other groups for needs related to sexual, gender and/or bodily diversity.
- Involving the Aboriginal Mental Health Liaison or Aboriginal Liaison Officers where appropriate to assist with cultural and spiritual needs
- Ensuring respect for privacy, dignity, and independence (including requirements for obtaining consent for treatment and release of information).
- Having clear and transparent discussions regarding roles and responsibilities with people who are receiving care and their support people. This includes expectations of both the service and the person receiving care in relation to treatment, support, and discharge planning.
- Maintaining regular contact through assertive outreach at agreed intervals.
- Seeking support and guidance from the MDT as required to provide optimal service. This may include a specific skill set or opinion from another discipline.
- Escalating issues to the MDT for discussion and decision regarding assertion required to engage and involve the person in their treatment, care, and support

7.2. Acting as a primary contact within the community mental health service

The Care Coordinator acts as the primary contact between the person receiving care and the mental health service, as well as the primary contact within the MDT on behalf of the person. This function is undertaken by:

- Providing a single point of contact for the person accessing mental health services and their support people.
- Enabling and fostering partnerships through identifying all support people that are involved in the care of the person.
- Engaging in discussion and planning regarding goals on entry to, and discharge from, an inpatient service.
- Maintaining contact and coordination during any inpatient (mental or general health) admission. This may include in-reach to support the person on the ward and/or attending the inpatient MDT meeting.
- Facilitating discussions regarding care and discharge planning within the Community Mental Health Service, with the MDT and at Clinical Review Meetings, including facilitating input from the person receiving care, their support people, GP, and other primary health care professionals. This may include escalation of issues / risks to the MDT and obtaining an expert opinion when required.
- Ensuring completion of all required documentation, as per the [WA Health Statewide Standardised Clinical Documentation for Mental Health \(SSCD\)](#).

- Ensuring collaborative review of the individual TSDP with the person at least every three months, to facilitate ongoing delivery of treatment, care, and support in line with the SSCD, Office of the Chief Psychiatrist Standards for Clinical Care, and the National Safety and Quality Health Service (NSQHS) Standards (second edition).
- Completion of outcome measures including Phase of care and National Outcome and Casemix Collection (NOCC) measures

7.3. Brokering of services

Facilitating referrals to any additional required services that support psycho-social, cultural, environmental, and sexual, gender and/or bodily diversity needs is a key element of holistic care planning. The Care Coordinator undertakes this by:

- Referring, or coordinating referrals, to other services and members of the MDT as required, including, but not limited to:
 - Non-Government Organisations (NGOs),
 - Community Managed Organisations (CMOs) for psycho-social support,
 - Traditional Healers for Aboriginal people,
 - National Disability Insurance Scheme (NDIS) to access psycho-social services,
 - Government agencies for family, housing, and financial support.
- Participating in shared assessments with additional services, where possible and with the consent of the person accessing care, to ensure collaborative service partnerships starts from point of referral.
- Supporting the person accessing care with their physical health care needs including linking with their General Practitioner (GP), including Aboriginal Community Controlled Health Services (ACCHSs), and attending medical appointments and other treatments/therapies when required/scheduled.
- Assisting and supporting the person receiving care to access other health services when indicated (e.g., vision, hearing, dental, podiatry, sexual health services).
- Assisting the person receiving care in obtaining legal representation when needed.
- Supporting follow-through with all outside referrals for benefits and services.

7.4. Advocacy and liaison

Care Coordinators, where applicable and supported by the person accessing care, may assume the role of advocate or liaison to ensure the persons views and wishes are heard and considered.

Advocacy may be undertaken by:

- Supporting the person receiving care with access to services
- Advocating within the MDT
- Working with and supporting the Mental Health Advocacy Service (MHAS), if the person is an identified person under the Mental Health Act 2014, ensuring their rights are maintained.
- Supporting the person receiving care in navigating housing, justice and other services/agencies where required
- Helping to safeguard the rights of the person in line with the [Mental Health Act 2014 Charter of Mental Health Care Principles](#). Further information can be found via the [Mental Health Advocacy Service](#).

Liaison roles may be undertaken by:

- Effective engagement in clinical handover processes to ensure explicit transfer of professional responsibility and accountability for some or all aspects of care, to another person or professional group on a temporary or permanent basis, as per the [WA Department of Health Clinical Handover Policy](#).
- Ensuring timely communication and liaison with services/individuals involved in the care of the person, including GP/ACCHSs, MDT members, inpatient teams (when relevant) and other agencies.
- Ensuring communication and liaison with support people.
- Acting as a liaison in mental or general health services when the person receiving care may require hospital admission.

7.5. Care planning

Development, and regular review, of the TSDP must be in collaboration with the person receiving care and their support people (where possible), in line with the [Statewide Standardised Clinical Documentation for Mental Health](#). Information in the TSDP will include, but not be limited to:

- What matters to the person, their needs, goals, current support, and treatment
- Views of the consumer, family, carer and/or PSP
- Safety planning and risk mitigation
- Outcomes measures including Phase of care and National Outcome and Casemix Collection (NOCC) measures
- Crisis and relapse prevention plans
- Physical health care needs and requirements (e.g., metabolic screening, Liverpool University Neuroleptic Side Effect Rating Scale [LUNSERS], ASSIST-Lite)
- Role and responsibilities of the person receiving care and the Care Coordinator
- Date of next clinical review
- Discharge planning, identification of planned discharge date and exploration of barriers to discharge

Best practice guidelines in relation to treatment options should be considered, as appropriate and per service Models of Care.

7.6. Therapeutic interventions

The Care Coordinator, in partnership with the MDT and acting within their defined scope of practice, will utilise evidence-based practice to provide a range of therapeutic interventions that are recovery-oriented and tailored to meet the needs of the person receiving care.

Therapeutic interventions must be delivered in alignment with the [Chief Psychiatrist's Standards for Clinical Care](#) and the [National framework for recovery-oriented mental health services](#).

Therapeutic support should be delivered in all phases of care.

7.7. Review / escalation of care

The Care Coordinator must communicate to the MDT any change in clinical presentation that requires escalation or review of care or intervention. Interventions will be based on comprehensive multidisciplinary assessment and collaborative, person-centred, care planning and will include, where applicable:

- Responsive and evidence-based treatments, as appropriate and per service Models of Care
- Trauma informed and recovery focused care.
- Collaboration with the person receiving care (and their support people if applicable) to identify possible coping strategies to assist with their individual situation.
- Monitoring effects of, and adherence to, TSDP including prescribed medication. This includes psychoeducation relative to any treatment plan.
- Promoting and supporting life skills development to empower independence.
- Psychoeducation and support regarding mental health concerns for the person receiving care and their support people.

7.8. Discharge planning

Discharge planning is integral to reducing risk and ensuring seamless transition of care (including to external agencies). Discharge planning should be commenced at the earliest opportunity after entry (or re-entry) to the service, and a Planned Discharge Date (PDD) discussed with the person accessing the service, to allow for development of a person-centred journey. The treating psychiatrist, Care Coordinator, and MDT undertake discharge planning by:

- Establishing expectations with the person receiving care and their support people regarding the potential length of treatment within the community program at commencement with the service, and at appropriate times throughout treatment, support, and discharge planning.
- Collaborating with the person receiving care and their support people regarding requirements for ongoing care and options, based on discussion with the MDT and relevant ongoing services where applicable (e.g., shared care where appropriate to treatment provision and service model of care).
- Providing information to the person receiving care about confidentiality and sharing information and required consent.
- Development of a TSDP in collaboration with the person receiving care and their support people, in line with phase of care and PDD.
- Development of a comprehensive Safety Plan with the person receiving care and their support people, as per the [Safety Planning for Mental Health Consumers policy](#).
- Phone contact and handover to ongoing service providers (including GPs) to support effective referrals.
- Discussing and providing written information regarding follow-up appointments, ongoing service providers and emergency care contact details to the person and their support people.
- Discussing and providing written information regarding re-entry to the service to the person and their support people, ensuring there is a clear pathway if required.
- Ensuring timely preparation and communication of discharge information, as per the [SMHS Discharge Summary Policy](#) and site-based procedures for discharge.

8. Care coordination and inpatient admission

Where a person receiving care is admitted to an inpatient unit, the Care Coordinator must work collaboratively with the inpatient treating team to support the provision of care in alignment with the care coordination policy, where possible.

8.1. Referral / admission

Where the Care Coordinator is the referring clinician, they must work with the inpatient team to facilitate the referral and admission process to support a smooth transfer of care. This includes the Care Coordinator providing handover to the inpatient treating team and engaging in discharge planning from admission. Documentation must be completed as per SSCD Triage to Discharge Pathway.

Where the Care Coordinator is not the referring clinician, the inpatient treating team must seek out and engage the Care Coordinator.

Where the person receiving care has an identified General Practitioner (GP) or engages with other relevant service providers, the inpatient treating team must ensure notification of admission and any changes to treatment planning

8.2. Inpatient admission

During the inpatient admission, the Care Coordinator must maintain engagement with the person receiving care and their treatment plan. This may include attending ward rounds, family meetings, and engaging in treatment planning.

The inpatient treating team must ensure engagement with the Care Coordinator throughout the admission, including timely communication to support attendance at relevant meetings. Where no Care Coordinator is identified, but will be required following discharge, a referral must be made to Community Mental Health Service for allocation of a Care Coordinator prior to discharge.

8.3. Discharge

The inpatient treating team must engage with the person receiving care, their support people and the Care Coordinator in discharge planning and coordination of ongoing care and treatment following discharge. Timely communication with the Community Mental Health team is required to ensure their capacity to participate in discharge and care planning for the person accessing the service.

Where a person has been referred for Community Mental Health follow up, the inpatient MDT must provide follow up appointment information (where possible), crisis and emergency contact and pathway information prior to discharge.

The inpatient team must alert the relevant Community Mental Health team prior to discharge and provide handover to the Care Coordinator to support smooth transition of care, including relevant documentation as per SSCD Triage to Discharge Pathway.

Where the person receiving care has an identified General Practitioner (GP) or engages with other relevant service providers, the inpatient treating team must ensure notification of discharge and any changes to treatment planning

8.4. Follow up

Seven day follow up should be completed as per site policies and procedures, relative to the inpatient setting.

Note: If concerns are identified from 7 day follow up, they must be communicated to the Care Coordinator / MDT by phone in a timely manner and documented in the medical record and PSOLIS.

9. Compliance monitoring

9.1. Compliance monitoring

SMHS Mental Health sites or Service Director are to ensure compliance with this policy.

Compliance monitoring methods may include:

- Number (n) and percentage (%) of people activated within the program:
 - who have an allocated Care Coordinator
 - with clinical review every three months
 - with completed TSDP
 - with a RAMP completed in required time period.
 - with an identified GP liaison service event.
 - with evidence of support people (family/carer) involvement, as per service events
- Number (n) and percentage (%) of completed National Outcome and Casemix Collection (NOCC) measures.
- Number of mandatory service event items per program.
- Compliance with 7-day follow up as per site procedures.
- Compliments and complaints

10. Legislation

- [Mental Health Act 2014](#)
- [Carers Recognition Act 2004 WA – Schedule 1](#)
- [Carer Recognition Act 2010 – Part 1 Section 5; Part 2 Section 8; Schedule 1](#)

10.1.

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

11. Related Documents

11.1.WA Health policies

- [MP 0051/17 Language Services Policy](#)

- [MP 0155/21 State-wide Standardised Clinical Documentation SSCD for Mental Health Services](#)
- [MP 0175/22 Consent to Treatment Policy](#)
- [MP 0063/17 Road based Transportation for Mental Health Consumers policy](#)

11.2. SMHS policies

- [SMHS:17 Acute Response Mental Health Emergencies / Crises Policy](#)
- [SMHS:14 Triage Assessment of Referrals in Community Mental Health Policy](#)
- [SMHS:8 Discharge Policy](#)
- [SMHS:7 Discharge Summary Policy](#)
- [SMHS:131 Community / Home Visiting Policy](#)
- [SMHS:36 Substantive Equality Policy](#)
- [SMHS:20 Transportation of Mental Health Consumers Policy](#)
- SMHS Person Centred Care Framework

11.3. Other documents

- [Chief Psychiatrist's Standards for Clinical Care](#)
- [WA Disability Health Framework 2015-2025](#)
- [Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation](#)

11.4. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 5.0	10 July 2025	4 July 2025	4 July 2028	July 2028	Full Review – Triannual

6.6. Approval

Approval by	Tim Leen – Executive Director Mental Health
Approval date	1 July 2025

12. Appendix A: SMHS MH Care coordination procedure

12.1. Referrals

Referrals to SMHS MH must have an initial assessment completed and where possible documented on the Mental Health Assessment and Risk Assessment and Management Plan, to determine the most appropriate care provider (based on acuity, complexity and risk). This should be completed as soon as practicable after referral (if not completed by the referrer) and the results communicated to the referrer, the referred individual and the carer or support persons.

- Referrals will be assessed and prioritised according to the requirements of the individual and the level of risk.
- Following receipt of the referral, a senior clinician is to be allocated (for example the duty officer or the triage officer).
- Where required (new patients) relevant clerical staff are to obtain a Unit Medical Record Number (UMRN) from the Patient Administration System (PAS).
- The senior clinician will complete a triage service event on the PSOLIS.
- Wherever possible, collateral information should be obtained to guide the decision-making process.
- All referrals will be discussed as part of routine multidisciplinary clinical intake practices with a nominated consultant psychiatrist or nominee.
- The referrer and the referred individual should be contacted to provide information on the progress of the referral. The carer or personal support person will also be contacted where appropriate.
- An interim management plan, including management of any identified risk, will be documented pending referral to the MDT and being activated. The senior clinician is to liaise with the referrer, referred individual, and others as required to implement the interim plans.
- All available information is provided to the consultant psychiatrist and/or MDT for discussion and decision making relating to the referral and record information on PSOLIS and in the medical notes.
- The allocated clinician has responsibility for the care coordination responsibilities at the individual's point of entry into the MHS and acceptance by the MDT.
- If the referral is not accepted, the referring agency and individual will be notified, and recommendations may be identified for more appropriate treatment options. Where appropriate the MHS will obtain consent to forward the referral to the identified service on behalf of the consumer.

12.1.1. Referral accepted

Upon completion of initial assessment, the MHS will:

- provide the referring provider/agency with written confirmation, a copy of which is kept in the consumer's medical record; or verbal confirmation which should be documented in the consumer's medical record; and
- create a patient medical record or recall any existing consumer's medical record from Medical Records.

12.1.2. Referral not accepted

- The referring provider/agency must be notified and reasons for not accepting the referral are to be given.
- Notification of non-acceptance is to be in writing.
- Record of the date, type, and time of communication to the provider/ agency is to be made in the patient's medical record. The referral, relevant information and status will be updated in PSOLIS.
- Communication of the non-acceptance is to be provided to the referred individual, detailing if there are any recommendations regarding more appropriate treatment options and services.

12.2. Assessment

Following the initial assessment a provisional diagnosis is made using the International Statistical Classification of Diseases and Related Health Problems ICD10-AM diagnostic tools and discussed with a senior mental health practitioner or psychiatrist. Formal diagnosis should only be made by clinicians who have this ability within their scope of practice.

All individuals who meet the ongoing admission criteria to community public mental health services will be allocated a Care Coordinator. The Care Coordinator will work collaboratively with the consumer, carer, and other relevant CMOs, where practicable, to complete:

- Mental Health Assessment
- Risk Assessment and Management Plan
- Physical Examination
- Physical Appearance
- Treatment Support and Discharge Plan (TSDP)
- Care Transfer Summary (where indicated)
- National Outcome Case mix Collection (NOCC) data undertaken at the time of admission to the service and subject to regular review.

12.3. Treatment and support

Treatment choices for mental health conditions vary from person to person. Even people with the same diagnosis will have different experiences, needs, goals and objectives for treatment. There is no "one size fits all" treatment. Choosing the right mix of treatments and supports that work for a person is an important step in the recovery process. When people are directly involved in designing their own treatment plan, including defining recovery and wellness goals, choosing services that support them and evaluating treatment decisions and progress, the experience of care and outcomes are improved.

Every consumer must have a current, comprehensive TSDP completed in webPSOLIS with a copy in the patient's medical record which is reviewed as a minimum every three months. Mandatory outcome measure results will be used to inform treatment and care planning.

The TSDP will be developed within the framework of the [Carer Recognition Act 2010](#), and should be person-centred, flexible, strengths based, culturally sensitive, recovery-focused and collaborative between all key stakeholders.

The consumer's individual TSDP will:

- be collaboratively developed by all stakeholders including the consumer with their consent
- be person-centred, flexible, strengths based, culturally sensitive, recovery-focused and collaborative between all key stakeholders
- be jointly signed by the consumer and where possible carers
- include a clear indication of agreed roles and responsibilities
- be updated according to the changing needs of the consumer at least every three months; and
- be documented in PSOLIS and a copy made available in the consumer's medical record.

A copy of the TSDP is to be provided to the consumer and the carer.

Physical health care needs, financial, social, employment and housing needs should be considered in conjunction with other agencies as part of holistic planning. Targeted interventions, discharge goals, treatment and support options must be discussed with the consumer and carers during the assessment process.

A Safety Plan which includes a description of early warning signs and relapse triggers will be completed with the consumer and/or carer where required.

12.4. Discharge / transfer of care and re-entry

When an individual is discharged from the community mental health service the Care Coordinator will ensure the following information is communicated and documented in the care transfer summary:

- the person and their carer/support persons know how to re-refer to the previous level of care through a re-assessment process
- the person and their carer/support persons are made aware that there are no barriers to re-refer
- whether a safety plan has been developed with the consumer and carer to recognise periods of crisis and increased need and to provide a plan of action to access help at these times
- effective coordination to enable continuity of care following discharge. This requires a rapid and effective handover and transmission of essential discharge information to carers, GPs and other relevant stakeholders.

12.4.1. Discharge / transfer of care to referrer / GP

Planning for discharge / transfer of care must begin at the point of entry into the service.

The Care Coordinator / allocated clinician will be responsible for liaison with the individual, carers and other support persons, GP and other stakeholders including referrer; and for supporting the discharge planning process at the earliest opportunity for seamless continuity of care.

- The discharge / transfer of care plan is to be agreed by the consumer, carer and the MDT.
- A deactivation form on PSOLIS is to be completed.
- A Care Transfer Summary stating current issues, interventions, medication regime, follow up services and recommendations is to be completed and provided to the consumer, carer, GP and all stakeholders who are partners in care.

- Phone contact should be made with the GP and/or referring agency if the care transfer summary is delayed.
- A copy of the Care Transfer Summary is to be filed in the consumer medical record.
- Consumers should be provided with the date and time of all appointments that have been arranged with services they have been referred to.
- The consumer be provided with the contact details of emergency services, out of hours contact numbers and other support services including the Mental Health Emergency Response Line, information on the process of re-entry to SMHS (or other health services) if needed, and the name of the mental health consumer's clinician or Care Coordinator.
- Consumers are only discharged / transferred when appropriate handover of all relevant information has occurred with the services the consumer is being referred to on discharge.
- If the consumer does not have a discharge contact or GP, then the mental health services will suggest a GP within the consumers catchment area.

All documentation must be maintained in accordance with site specific policies and procedures.

12.4.2. Discharge / transfer of care to a CMO

When a consumer is discharged to the ongoing care of a CMO, the Care Coordinator / allocated clinician is to ensure that relevant documentation is forwarded to CMO and that the consumer's medical record reflects these actions.

The Care Coordinator / allocated clinician is to ensure that:

- a joint hand over visit is initiated wherever possible to allow the new Care Coordinator the opportunity to introduce themselves to the consumer, or at a minimum telephone contact is to be initiated with the receiving agency prior to the consumer's discharge / transfer of care
- a Care Transfer Summary is completed prior to the discharge date and forwarded to the receiving agency at the time of discharge to ensure that the receiving agency has immediate access to the consumer's information
- as part of the discharge process, a follow up phone call is made by the Care Coordinator / allocated clinician to the receiving service to ensure that follow-up has occurred
- an entry is made in the consumer's medical record to evidence actions completed.

Where a referral to a CMO is not accepted, the Care Coordinator / allocated clinician is to meet or liaise with the consumer to explain their referral has been unsuccessful and to explore other options and make suitable recommendations.

12.4.3. Re-entry

Re-entry to the service once the consumer has been discharged is to be through the existing triage process following re-referral and assessment.

This document can be made available in alternative formats on request.

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